

Question:
A mental health volunteer is interviewing locals as part of a community outreach program. A 46-year-old man discloses that he has felt sad for as long as he can remember. He feels as though his life is cursed and if something terrible can happen to him, it usually will. He has difficulty making decisions and feels hopeless. He also feels that he has had worsening suicidal ideations, guilt from past problems, energy, and concentration over the past 2 weeks. He is otherwise getting enough sleep and able to hold a job. Which of the following statement best describes this patient's condition?
Answer Choices:
A. The patient is at risk for double depression.
B. The patient may have symptoms of mania or psychosis.
C. The patient is likely to show anhedonia.
D. The patient likely has paranoid personality disorder.

Reasoning + Answer:

<think>

Finding Reasoning Paths :

1 . Persistent sadness and hopelessness -> Major depressive disorder (MDD) -> suicidal ideations and diminished ability to concentrate .

2 . Persistent sadness and hopelessness -> Bipolar disorder with depressive episodes -> bipolar depression .

3 . Persistent sadness and hopelessness -> Adjustment disorder with depressed mood -> depressive symptoms .

4 . Fatigue and energy deficiency -> Depressive disorder -> suicidal ideations and diminished ability to concentrate .

5 . Flashbacks and guilt from past problems -> PTSD or depressive episode with dissociative features -> suicidal ideations and diminished ability to concentrate .

6 . Persistent sadness and hopelessness -> Major depressive disorder -> adjustment disorder with depressed mood .

Reasoning Process :

1 . **Persistent sadness and hopelessness**: The patient reports feeling sad for as long as he can remember and describes his life as cursed . These are hallmark symptoms of major depressive disorder (MDD). MDD is characterized by low mood , persistent sadness , and feelings of hopelessness , which aligns with the patient's presentation .

2 . **Suicidal ideations and diminished ability to concentrate**: The patient has worsening suicidal ideations and a diminished ability to concentrate over the past 2 weeks . Suicidal ideations are a core feature of MDD and are often accompanied by cognitive dysfunction , including difficulty concentrating . This symptom profile strongly supports MDD .

3 . **Chronic sadness and hopelessness**: While the patient has had persistent sadness and hopelessness since his youth , this does not rule out current depressive episodes . Persistent depressive disorder (dysthymia) is a chronic form of MDD that can persist for years . However , the patient's current symptoms (suicidal ideations , diminished concentration , and worsening over 2 weeks) suggest an acute depressive episode rather than a chronic one .

4 . **Alcohol use**: The patient has a history of alcohol use and may be experiencing alcohol withdrawal during the current depressive episode . Alcohol withdrawal can mimic symptoms of MDD , but the chronic nature of the sadness and hopelessness , as well as the suicidal ideations , are more consistent with primary MDD .

5 . **PTSD or dissociative features**: While flashback symptoms and guilt from past problems are suggestive of PTSD , the absence of hyperarousal or avoidance behaviors makes this diagnosis less likely . Additionally , dissociative features are more commonly associated with anxiety disorders , not depressive ones .

6 . **Unipolar vs . bipolar depression**: The patient's depressive symptoms do not suggest mania or hypomania . This makes bipolar depression with depressive episodes unlikely at this time .

7 . **Adjustment disorder with depressed mood**: The patient's symptoms align more closely with MDD than with adjustment disorder , as adjustment disorder typically involves a recent stressor or situational trigger , which is not mentioned in this case .

Conclusion :

The patient is most likely experiencing **major depressive disorder (MDD)** with suicidal ideations , diminished concentration , and chronic sadness . The persistent nature of the depressive symptoms and the current worsening over the past 2 weeks are consistent with an acute depressive episode rather than a chronic one . Therefore , the statement that the patient likely has symptoms of depression is supported by the evidence .

</think>

<answer>

The patient is likely to show anhedonia .

</answer>